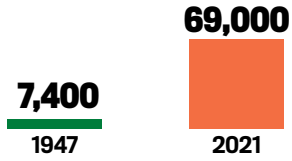
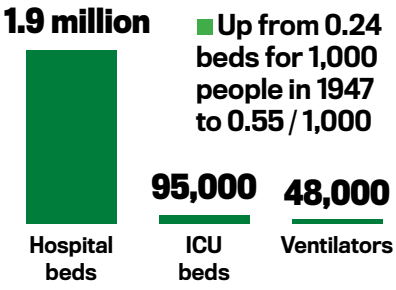
UNITS OF LIFE

NO. OF HOSPITALS



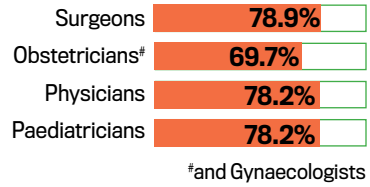
HOSPITAL BEDS



PRIMARY CARE CENTRES



SHORTFALL IN RURAL AREAS



#and Gynaecologists

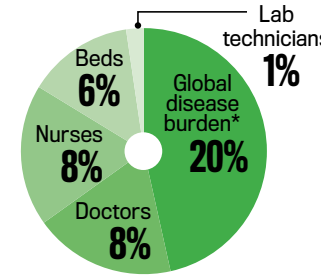
** Mizoram only state with no shortfall. Uttar Pradesh, Bihar and Jharkhand faced an acute deficit of medical staff even at PHC level. Sources: Central Bureau of Health Intelligence; Rural Health Statistics, 2019-20, Union health ministry assessment of 18 states*

81.2%
The percentage of 'allopathic doctors' in rural India in 2001 who were unqualified

31%
of the total rural doctor force in 2016 were educated only up to secondary school level

HOW WE COMPARE

Seen relatively on a global scale, our standards remain low. India is 145th among 195 countries in terms of healthcare access and quality, behind China (48), Sri Lanka (71), Bangladesh (133).



*(***India's share,** from the Global Burden of Disease study, 2019. India also had the widest disparity between regions in an analysis of seven nations–Brazil, China, England, Japan, Mexico and the US)*

DISEASE ERADICATION

MALARIA

2-3%
Global disease burden

Annual death toll: **1 million** (1947) → **77** (2021)

83.34%
reduction in morbidity, 92% in mortality in 2000-2019

SMALL POX

Eradicated in April 1977

TB

26% **Global** share

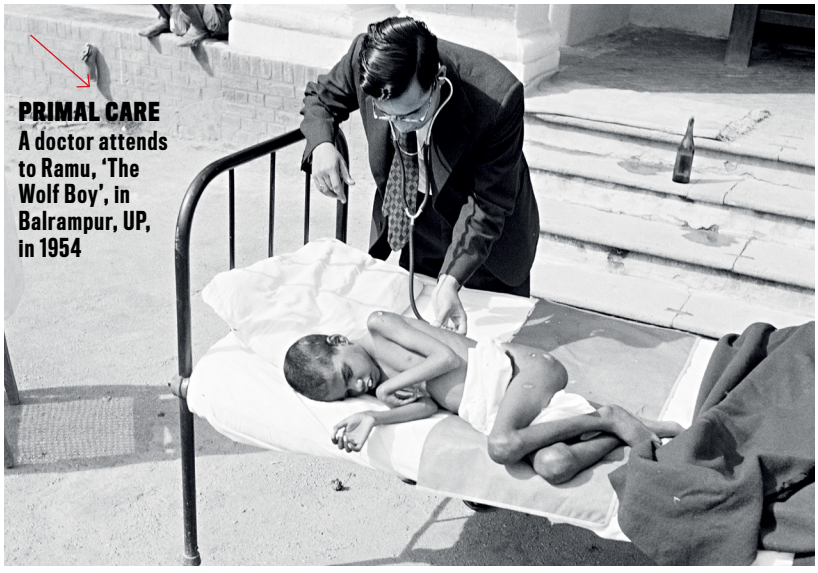
2025 **Target year** for total eradication

POLIO

January 13, 2011: Last case reported from Howrah, Bengal

Universal Immunisation Programme: 76% children fully immunised, finds NFHS-5, big jump from 62% in NFHS-4

2,083,124,694
Total Covid vaccinations as on Aug. 15, 2022



PRIMAL CARE
A doctor attends to Ramu, 'The Wolf Boy', in Balrampur, UP, in 1954

GETTY IMAGES

the importance of investment in health infrastructure. At a mere 1.2 per cent of the GDP, our health budget ranks among the world's lowest, even after public investment increased by 50.5 per cent between 2017-18 and 2021-22—from Rs 47,353 crore to Rs 71,269 crore. The budget estimate for 2022-23 is Rs 83,000 crore. Attaining 2.5-3 per cent of the GDP is a crucial first step to closing in on the world average of 6 per cent (China is at 5 per cent)—that would already ease out-of-pocket expenditure by 30 per cent. In recent years, the growing burden of cancer, TB and lifestyle diseases has distended that figure to an unhealthy 65 per cent (global average: 18.5 per cent). The curative needle must aim at both: sickness and the cost of health. For everyone.

To accomplish that, we must turn to the human component at